

NEW YORK INSTITUTE OF TECHNOLOGY

College of Osteopathic
Medicine

DPR: History and Physical Exam Pediatrics Part 2: 6 – 10 years old

PPOM1

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Learning Objectives:

1. Basic Well Child Visit Interviewing/History Taking
2. Growth
3. AAP recommendations
4. Psych Screenings
5. Vaccine Schedule
6. Likely pathologies
7. Scoliosis testing
8. Tanner Staging
9. Resources

Well child visits:

Ask about school, grades

Ask about screen time

Ask about Diet/Exercise

Ask about activities, sports participation, etc

Establishing good sleeping habits (sleeping about 9 hours a night)

Dentist visits, brushing twice a day!

Visual screening – need for glasses?

AAP recommends **1 random screening between 9-11 years** of age of lipids

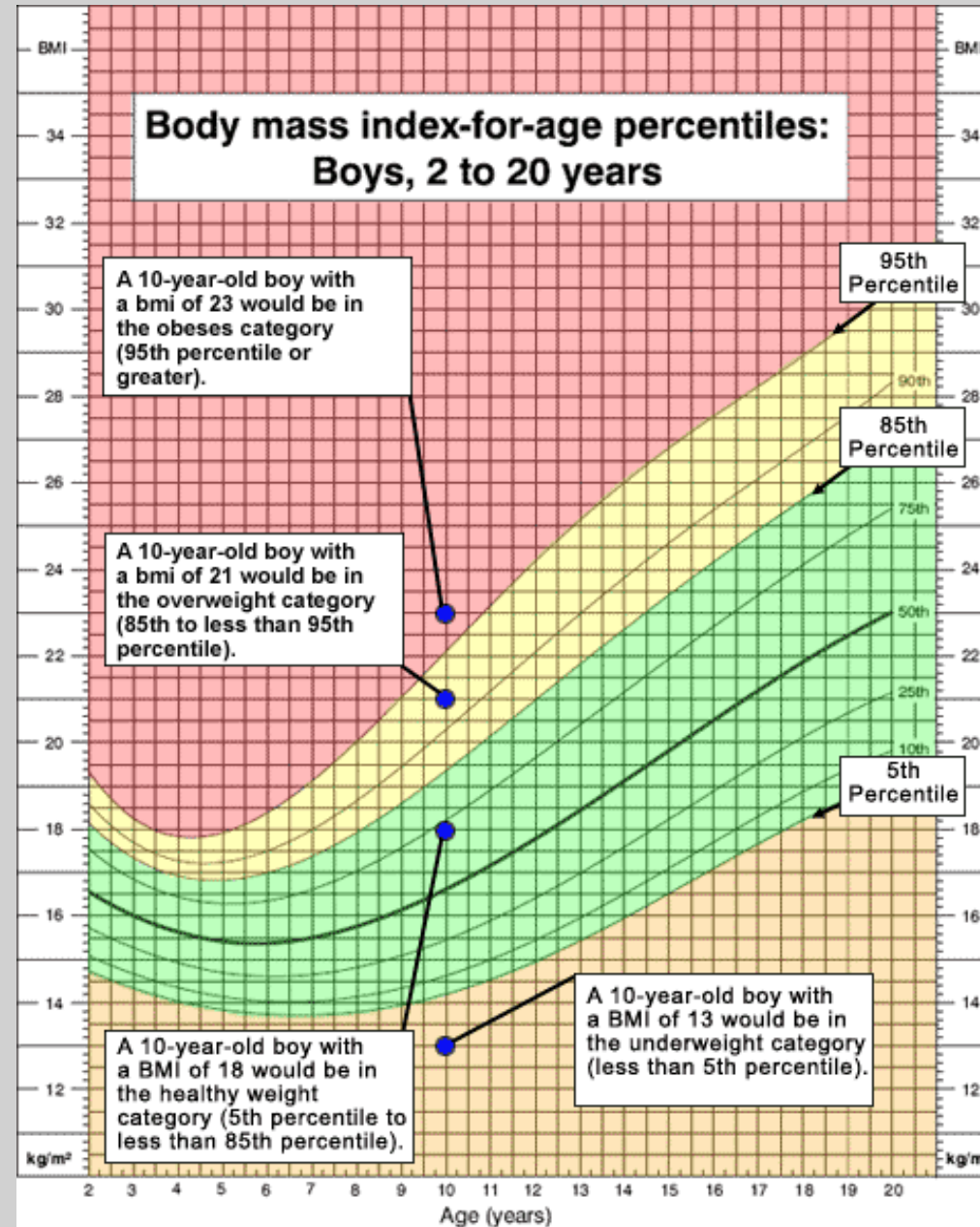
Cardiac screening questions (LOC while exercising, chest pain, early cardiac death in family)

Obesity (BMI %tile over 95 for age and sex) is a growing problem:

1 in 5 US children and adolescents suffer from obesity



Keep checking
your growth charts!



Updated Definitions of Blood Pressure Categories and Stages^[52]

	For Children Ages 1 to <13 years	For Children Ages ≥13 years
Normal	<90th percentile	<120/<80 mm Hg
Elevated	≥90th percentile to <95th percentile or 120/80 mm Hg to <95th percentile (whichever is lower)	120/<80 to 129/<80 mm Hg
Stage 1 hypertension	≥95th percentile to <95th percentile + 12 mm Hg, or 130/80 to 139/89 mm Hg (whichever is lower)	130/80 to 139/89 mm Hg
Stage 2 hypertension	≥95th percentile + 12 mm Hg, or ≥140/90 mm Hg (whichever is lower)	≥140/90 mm Hg

Source: Reproduced with permission from Flynn JT, Kaelber DC, Baker-Smith CM, et al. Clinical practice guideline for screening and management of high blood pressure in children and adolescents. *Pediatrics*. 2017;140(3):e20171904. Copyright © 2017 American Academy of Pediatrics.

Average Heart Rate of Children at Rest^[36]

Age (Years)	Average Rate (Median)	Range (1st to 99th percentile)
1-2	110-120	88-155
2-6	100-110	65-140
6-10	75-90	52-130

Source: Fleming S, Thompson M, Stevens R, et al. Normal ranges of heart rate a observational studies. *Lancet*. 2011;377(9770):1011-1018.

Keep an eye on length as well

Short Stature:

Constitutional Growth Delay

Familial

Pathologic

Idiopathic

Constitutional delay: catch up growth will occur,
bone age young

Familial: calculating mid-parental height

Pathologic: GH deficiency, malnutrition causes
(weight will also be affected)

Idiopathic: tests normal

Table 1. Formulas for Calculating Midparental Height in Children

Girls

$$[\text{Paternal height (cm)} - 13 \text{ cm} + \text{maternal height (cm)}] \div 2$$

or

$$[\text{Paternal height (in)} - 5 \text{ in} + \text{maternal height (in)}] \div 2$$

Boys

$$[\text{Paternal height (cm)} + 13 \text{ cm} + \text{maternal height (cm)}] \div 2$$

or

$$[\text{Paternal height (in)} + 5 \text{ in} + \text{maternal height (in)}] \div 2$$



Bright Futures/American Academy of Pediatrics



These recommendations represent a consensus by the American Academy of Pediatrics (AAP) and Bright Futures. The AAP continues to emphasize the great importance of continuity of care in comprehensive health supervision and the need to avoid fragmentation of care.

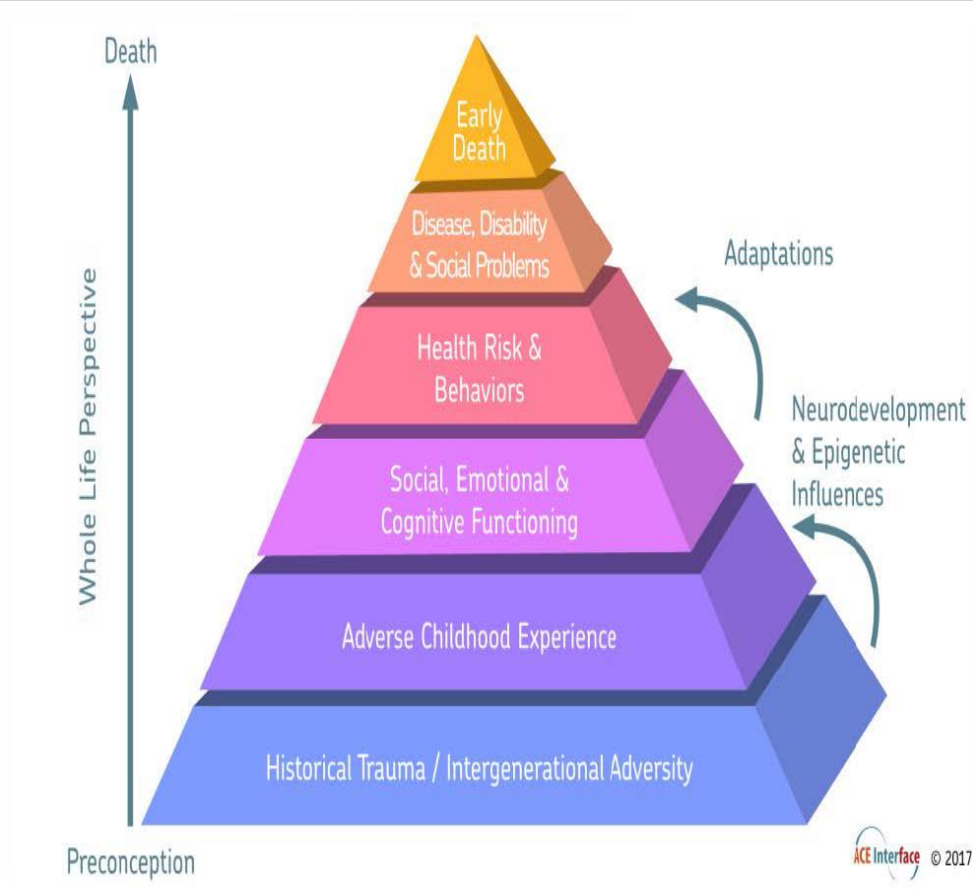
The recommendations in this statement do not indicate an exclusive course of treatment or serve as a standard of medical care. Variations, taking into account individual circumstances, may be appropriate.

The Bright Futures/American Academy of Pediatrics Recommendations for Preventive Pediatric Health Care are updated annually.

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	INFANCY								EARLY CHILDHOOD							MIDDLE CHILDHOOD						ADOLESCENCE										
AGE ¹	Prenatal ²	Newborn ³	3-5 d ⁴	By 1 mo	2 mo	4 mo	6 mo	9 mo	12 mo	15 mo	18 mo	24 mo	30 mo	3y	4 y	5y	6y	7y	8y	9y	10 y	11 y	12y	13y	14y	15y	16y	17y	18y	19y	20y	21y
HISTORY Initial/Interval	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●
MEASUREMENTS																																
Length/Height and Weight		●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●
Head Circumference		●	●	●	●	●	●	●	●	●	●	●																				
Weight for Length		●	●	●	●	●	●	●	●	●	●	●																				
Body Mass Index ⁵												●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●
Blood Pressure ⁶		★	★	★	★	★	★	★	★	★	★	★	★	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●
SENSORY SCREENING																																
Vision ⁷		★	★	★	★	★	★	★	★	★	★	★	★	●	●	●	●	★	●	★	●	★	★	●	★	★	●	★	★	★	★	★
Hearing		● ⁸	● ⁹	→	→	★	★	★	★	★	★	★	★	★	●	●	●	★	●	★	●	●	←	←	● ¹⁰	→	←	←	→	←	←	→
DEVELOPMENTAL/SOCIAL/BEHAVIORAL/MENTAL HEALTH																																
Maternal Depression Screening ¹¹				●	●	●	●																									
Developmental Screening ¹²								●			●		●																			
Autism Spectrum Disorder Screening ¹³											●	●																				
Developmental Surveillance		●	●	●	●	●	●	●	●	●	●	●		●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●
Behavioral/Social/Emotional Screening ¹⁴		●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●
Tobacco, Alcohol, or Drug Use Assessment ¹⁵																						★	★	★	★	★	★	★	★	★	★	★
Depression and Suicide Risk Screening ¹⁶																						●	●	●	●	●	●	●	●	●	●	●
PHYSICAL EXAMINATION ¹⁷		●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●
PROCEDURES ¹⁸																																
Newborn Blood		● ¹⁹	● ²⁰	→	→																											
Newborn Bilirubin ²¹		●																														
Critical Congenital Heart Defect ²²		●																														
Immunization ²³		●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●
Anemia ²⁴						★			●	★	★	★	★	★	★	★	★	★	★	★	★	★	★	★	★	★	★	★	★	★	★	★
Lead ²⁵							★	★	● or ★ ²⁶		★	● or ★ ²⁵		★	★	★	★	★	★	★	★	★	★	★	★	★	★	★	★	★	★	★
Tuberculosis ²⁷				★			★		★					★	★	★	★	★	★	★	★	★	★	★	★	★	★	★	★	★	★	★
Dyslipidemia ²⁸												★			★		★		★		←	←	●	→	★	★	★	★	★	★	★	→
Sexually Transmitted Infections ²⁹																						★	★	★	★	★	★	★	★	★	★	★
HIV ³⁰																						★	★	★	★	●						→
Hepatitis B Virus Infection ³¹		★																														→
Hepatitis C Virus Infection ³²																																→
Sudden Cardiac Arrest/Death ³³																																→
Cervical Dysplasia ³⁴																						★										●
ORAL HEALTH ³⁵							● ³⁶	● ³⁶	★		★	★	★	★	★	★	★															
Fluoride Varnish ³⁷							←				●					→																
Fluoride Supplementation ³⁸								★	★	★	★	★	★	★	★	★	★	★	★	★	★	★	★	★	★	★	★	★	★	★	★	★
ANTICIPATORY GUIDANCE	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●	●

Adverse Childhood Experiences (ACE) screening



What Impact Do ACEs Have?

As the number of ACEs increases, so does the risk of negative health outcomes



Possible Risk Outcomes:

BEHAVIOR



Lack of Physical Activity



Smoking



Alcoholism



Substance Abuse



Missed Work

PHYSICAL & MENTAL HEALTH



Severe Obesity



Diabetes



Depression



Suicide Threats



STIs



Heart Disease



Cancer



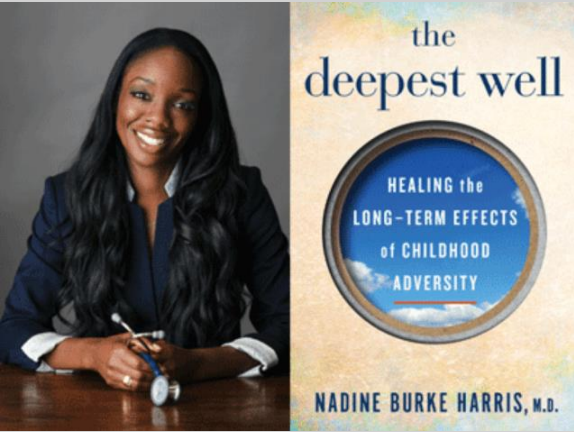
Stroke



COPD



Broken Bones



Adverse Childhood Experiences (ACE) 10-Question Survey

PRIOR TO YOUR 18th BIRTHDAY:

1. Did a parent or other adult in the household **often or very often**...
Swear at you, insult you, put you down, or humiliate you? OR
Act in a way that made you afraid that you might be physically hurt? If YES, enter 1 _____
2. Did a parent or other adult in the household **often or very often**...
Push, grab, slap, or throw something at you? OR
Ever hit you so hard that you had marks or were injured? If YES, enter 1 _____
3. Did an adult or person at least 5 years older than you **ever**...
Touch or fondle you or have you touch their body in a sexual way? OR
Attempt or actually have oral, anal, or vaginal intercourse with you? If YES, enter 1 _____
4. Did you **often or very often** feel that...
No one in your family loved you or thought you were important or special? OR
Your family didn't look out for each other, feel close to each other,
or support each other? If YES, enter 1 _____
5. Did you **often or very often** feel that...
You didn't have enough to eat, had to wear dirty clothes, and had no one to protect you? OR
Your parents were too drunk or high to take care of you or
take you to the doctor if you needed it? If YES, enter 1 _____
6. Was a biological parent **ever** lost to you through divorce, abandonment,
or other reason? If YES, enter 1 _____
7. Was your mother or stepmother:
Often or very often pushed, grabbed, slapped, or had something thrown at her? OR
Sometimes, often, or very often kicked, bitten, hit with a fist, or hit with something hard?
OR
Ever repeatedly hit over at least a few minutes or threatened
with a gun or knife? If YES, enter 1 _____
8. Did you live with anyone who was a problem drinker or alcoholic or
who used street drugs? If YES, enter 1 _____
9. Was a household member depressed or mentally ill or did a household
member attempt suicide? If YES, enter 1 _____
10. Did a household member go to prison? If YES, enter 1 _____

Now add up your YES answers – **TOTAL "Yes":** _____

This is your ACE score

- Depression screening PHQ2 >10 age
- IF POS further with PHQ9

TABLE 2

PHQ-2 Screening Instrument for Depression

Over the past two weeks, how often have you been bothered by any of the following problems?	Not at all	Several days	More than half the days	Nearly every day
Little interest or pleasure in doing things	0	1	2	3
Feeling down, depressed, or hopeless	0	1	2	3

Scoring: A score of 3 or more is considered a positive result. The PHQ-9 (Table 3) or a clinical interview should be completed for patients who screen positive.

PHQ = Patient Health Questionnaire.

Adapted from Patient Health Questionnaire (PHQ) screeners. <http://www.phqscreeners.com>. Accessed February 8, 2018.

PATIENT HEALTH QUESTIONNAIRE (PHQ-9)

Question	Score	Soto 1	prippy
1 Little interest or pleasure in doing things	0		
2 Feeling down, depressed, or hopeless	1		
3 Trouble falling/staying asleep, or sleeping too much	2		
4 Feeling tired or having little energy	5		
5 Poor appetite or oversating	6		
6 Feeling bad about yourself — or that you are a failure	7		
7 Trouble concentrating on things	8		
8 Moving/speaking so slowly that other people could have noticed — or the opposite — being so fidgety or restless that you have been moving around a lot more than usual	8		
9 Thoughts that you would be better off dead, or of hurting yourself	2		
Total Score			
Scoring: 6-41 6-41 Mild depression 20-27 ~ Moderate to severe 0-5 14-11 Mild Severe ▼ 13-19 Moderately severe			

Print with Purpose C

PHQ-9 QUESTIONNAIRE DEPRESSION SCREENING PRINTABLE PDF

ADHD (attention deficit hyperactivity disorder): is a common, chronic, pervasive, neurodevelopmental disorder characterized by developmentally inappropriate levels of hyperactivity, impulsivity, and inattention that adversely affect behavioral, emotional, cognitive, academic, occupational, and social functions

We use a Vanderbilt questionnaire to assess whether similar behaviors in 2 or more settings (school, home, after school place, work, play)

Inattention: more common in F, only inattention criteria met

Hyperactivity: more common in M, only hyperactivity criteria met

Combined: more common in M, both

At least several inattentive or hyperactive-impulsive symptoms occur **before age 12 years**

Strong genetic component but multifactorial

Hyperactivity/impulsivity symptoms usually appear by age 4 years and peak by age 7 or 8 years

Prominent symptoms are motor restlessness and disruptive behavior

Inattention symptoms usually appear by age 8 or 9 years

Older kids: usually inattention, hyperactivity is less pronounced

Can be mild, moderate to severe based on degree of impairment

Rule out: age appropriate behavior, anxiety/depression, learning disabilities, sleep disturbances, chronic illness, substance use, bipolar

Treatment: multi-modal (stimulant/nonstimulant medications plus behavior therapy) most beneficial – IEP (individualized education program) – Multimodal

NICHQ Vanderbilt Assessment Scale—PARENT Informant

Today's Date: _____ Child's Name: _____ Date of Birth: _____

Parent's Name: _____ Parent's Phone Number: _____

Directions: Each rating should be considered in the context of what is appropriate for the age of your child.

When completing this form, please think about your child's behaviors in the past 6 months.

Is this evaluation based on a time when the child ☐ was on medication ☐ was not on medication ☐ not sure?

Symptoms	Never	Occasionally	Often	Very Often
1. Does not pay attention to details or makes careless mistakes with, for example, homework	0	1	2	3
2. Has difficulty keeping attention to what needs to be done	0	1	2	3
3. Does not seem to listen when spoken to directly	0	1	2	3
4. Does not follow through when given directions and fails to finish activities (not due to refusal or failure to understand)	0	1	2	3
5. Has difficulty organizing tasks and activities	0	1	2	3
6. Avoids, dislikes, or does not want to start tasks that require ongoing mental effort	0	1	2	3
7. Loses things necessary for tasks or activities (toys, assignments, pencils, or books)	0	1	2	3
8. Is easily distracted by noises or other stimuli	0	1	2	3
9. Is forgetful in daily activities	0	1	2	3
10. Fidgets with hands or feet or squirms in seat	0	1	2	3
11. Leaves seat when remaining seated is expected	0	1	2	3
12. Runs about or climbs too much when remaining seated is expected	0	1	2	3
13. Has difficulty playing or beginning quiet play activities	0	1	2	3
14. Is "on the go" or often acts as if "driven by a motor"	0	1	2	3
15. Talks too much	0	1	2	3
16. Blurts out answers before questions have been completed	0	1	2	3
17. Has difficulty waiting his or her turn	0	1	2	3
18. Interrupts or intrudes in on others' conversations and/or activities	0	1	2	3
19. Argues with adults	0	1	2	3
20. Loses temper	0	1	2	3
21. Actively defies or refuses to go along with adults' requests or rules	0	1	2	3
22. Deliberately annoys people	0	1	2	3
23. Blames others for his or her mistakes or misbehaviors	0	1	2	3
24. Is touchy or easily annoyed by others	0	1	2	3

NICHQ Vanderbilt Assessment Scale: Teacher Informant

Teacher's Name: _____

Class Time: _____

Today's Date: _____

Child's Name: _____

Class Name/Period: _____

Grade Level: _____

Directions: Each rating should be considered in the context of what is appropriate for the age of the child you are rating and should reflect that child's behavior since the beginning of the school year.

Please indicate the number of weeks or months you have been able to evaluate the behaviors: _____

Is this evaluation based on a time when the child ☐ was on medication ☐ was not on medication ☐ not sure?

Symptoms	Never	Occasionally	Often	Very Often
1. Fails to give attention to details or makes careless mistakes in schoolwork	☐ 1	☐ 2	☐ 3	☐ 4
2. Has difficulty sustaining attention to tasks or activities	☐ 1	☐ 2	☐ 3	☐ 4
3. Does not seem to listen when spoken to directly	☐ 1	☐ 2	☐ 3	☐ 4
4. Does not follow through on instructions and fails to finish schoolwork (not due to oppositional behavior or failure to understand)	☐ 1	☐ 2	☐ 3	☐ 4
5. Has difficulty organizing tasks and activities	☐ 1	☐ 2	☐ 3	☐ 4
6. Avoids, dislikes, or is reluctant to engage in tasks that require sustained mental effort	☐ 1	☐ 2	☐ 3	☐ 4
7. Loses things necessary for tasks or activities (school assignments, pencils, or books)	☐ 1	☐ 2	☐ 3	☐ 4
8. Is easily distracted by extraneous stimuli	☐ 1	☐ 2	☐ 3	☐ 4
9. Is forgetful in daily activities	☐ 1	☐ 2	☐ 3	☐ 4
10. Fidgets with hands or feet or squirms in seat	☐ 1	☐ 2	☐ 3	☐ 4
11. Leaves seat in classroom or in other situations in which remaining seated is expected	☐ 1	☐ 2	☐ 3	☐ 4
12. Runs about or climbs excessively in situations in which remaining seated is expected	☐ 1	☐ 2	☐ 3	☐ 4
13. Has difficulty playing or engaging in leisure activities quietly	☐ 1	☐ 2	☐ 3	☐ 4
14. Is "on the go" or often acts as if "driven by a motor"	☐ 1	☐ 2	☐ 3	☐ 4
15. Talks excessively	☐ 1	☐ 2	☐ 3	☐ 4
16. Blurts out answers before questions have been completed	☐ 1	☐ 2	☐ 3	☐ 4
17. Has difficulty waiting in line	☐ 1	☐ 2	☐ 3	☐ 4
18. Interrupts or intrudes on others (eg, butts into conversations/games)	☐ 1	☐ 2	☐ 3	☐ 4
19. Loses temper	☐ 1	☐ 2	☐ 3	☐ 4
20. Actively defies or refuses to comply with adult's requests or rules	☐ 1	☐ 2	☐ 3	☐ 4
21. Is angry or resentful	☐ 1	☐ 2	☐ 3	☐ 4
22. Is spiteful and vindictive	☐ 1	☐ 2	☐ 3	☐ 4
23. Bullies, threatens, or intimidates others	☐ 1	☐ 2	☐ 3	☐ 4
24. Initiates physical fights	☐ 1	☐ 2	☐ 3	☐ 4
25. Lies to obtain goods for favors or to avoid obligations (eg, "cons" others)	☐ 1	☐ 2	☐ 3	☐ 4
26. Is physically cruel to people	☐ 1	☐ 2	☐ 3	☐ 4
27. Has stolen items of nontrivial value	☐ 1	☐ 2	☐ 3	☐ 4
28. Deliberately destroys others' property	☐ 1	☐ 2	☐ 3	☐ 4
29. Is fearful anxious or worried	☐ 1	☐ 2	☐ 3	☐ 4
30. Is self-conscious or easily embarrassed	☐ 1	☐ 2	☐ 3	☐ 4
31. Is afraid to try new things for fear of making mistakes	☐ 1	☐ 2	☐ 3	☐ 4
32. Feels worthless or inferior	☐ 1	☐ 2	☐ 3	☐ 4
33. Blames self for problems; feels guilty	☐ 1	☐ 2	☐ 3	☐ 4
34. Feels lonely, unwanted, or unloved; complains that "no one loves him or her"	☐ 1	☐ 2	☐ 3	☐ 4
35. Is sad unhappy, or depressed	☐ 1	☐ 2	☐ 3	☐ 4

Table 1

Recommended Child and Adolescent Immunization Schedule for Ages 18 Years or Younger, United States, 2025

American Academy of Pediatrics
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These recommendations must be read with the **Notes** that follow. For those who fall behind or start late, provide catch-up vaccination at the earliest opportunity as indicated by the outlined purple bars. To determine minimum intervals between doses, see the catch-up schedule (Table 2).

Vaccine and other immunizing agents	Birth	1 mos	2 mos	4 mos	6 mos	8 mos	9 mos	12 mos	15 mos	18 mos	19–23 mos	2–3 yrs	4–6 yrs	7–10 yrs	11–12 yrs	13–15 yrs	16 yrs	17–18 yrs		
Respiratory syncytial virus (RSV-mAb [nirsevimab, clesrovimab])	1 dose during RSV season depending on maternal RSV vaccination status (See Notes)					1 dose nirsevimab during RSV season (See Notes)														
Hepatitis B (HepB)	1 st dose	2 nd dose			3 rd dose															
Rotavirus (RV): RV1 (2-dose series), RVS (3-dose series)			1 st dose	2 nd dose	See Notes															
Diphtheria, tetanus, and acellular pertussis (DTaP <7 yrs)			1 st dose	2 nd dose	3 rd dose				4 th dose			5 th dose								
Haemophilus influenzae type b (Hib)			1 st dose	2 nd dose	See Notes				3 rd or 4 th dose (See Notes)											
Pneumococcal conjugate (PCV15, PCV20)			1 st dose	2 nd dose	3 rd dose				4 th dose											
Inactivated poliovirus (IPV)			1 st dose	2 nd dose	3 rd dose								4 th dose	See Notes						
COVID-19 (1vCOV-mRNA, 1vCOV-aPS)					1 or more doses of 2025–2026 vaccine (See Notes)								1 dose of 2025–2026 vaccine (See Notes)							
												1 dose of 2025–2026 vaccine (See Notes)								
Influenza					1 or 2 doses annually (See Notes)										1 dose annually (See Notes)					
Measles, mumps, and rubella (MMR)					See Notes			1 st dose				2 nd dose								
Varicella (VAR)								1 st dose				2 nd dose								
Hepatitis A (HepA)					See Notes			2-dose series (See Notes)												
Tetanus, diphtheria, and acellular pertussis (Tdap ≥7 yrs)															1 st dose					
Human papillomavirus (HPV)															2-dose series	See Notes				
Meningococcal (MenACWY-CRM ≥2 mos, MenACWY-TT ≥2years)			See Notes														1 st dose		2 nd dose	
Meningococcal B (MenB-4C, MenB-FHbp)															See Notes					
Respiratory syncytial virus vaccine (RSV [Abrysvo])															Seasonal administration during pregnancy if not previously vaccinated					
Dengue (DEN4CYD: 9–16 yrs)															Seropositive in areas with endemic dengue (See Notes)					
Mpox																				



Range of recommended ages for all children



Range of recommended ages for catch-up vaccination



Range of recommended ages for certain high-risk groups or populations



Recommended vaccination for those who desire protection



Recommended vaccination based on shared clinical decision-making

Vaccine Schedule

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In this age range, likely pathologies

Strep pharyngitis

peak incidence age 5-9

Use of Centor Criteria

Swab: Rapid Strep + Culture

Treatment: Beta-lactams, Amoxil

vs viral pharyngitis: supportive

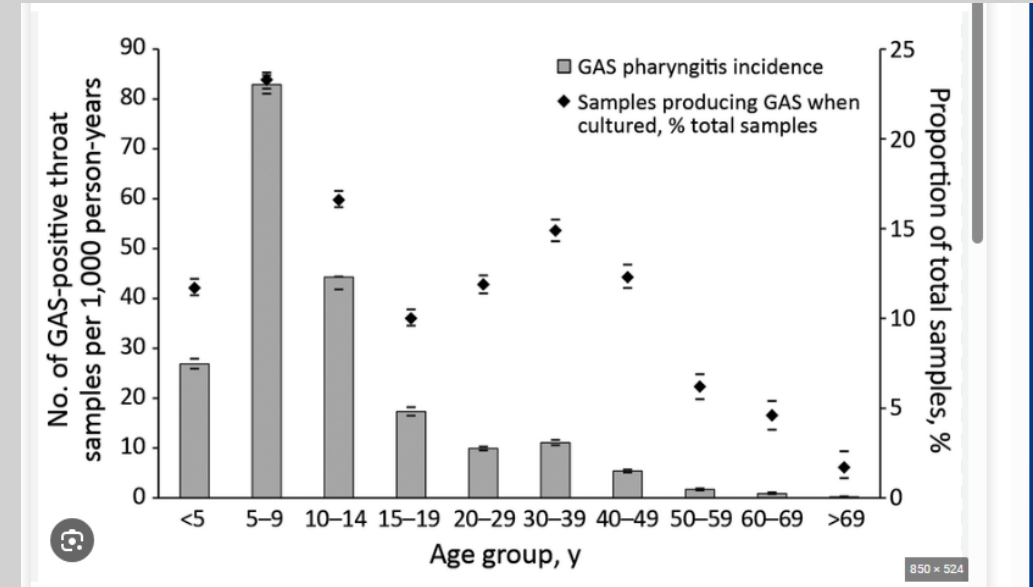


Table 1. Modified/McIsaac Centor Criteria³⁴

Criteria	Points
Age 3-14 years	1
Tonsillar swelling or exudates	1
Tender/swollen anterior cervical lymph nodes	1
Fever (temperature > 38°C [100.4°F])	1
Absence of cough	1
Total	_____

0 or 1 point = Group A *Streptococcus* (GAS) testing not usually required.

≥ 2 points = GAS testing usually required.



In this age range, likely pathologies

- During oral exam:
Make sure you look at dentition
- Getting adult teeth
- Prescence
of Dental caries

Tooth Types and Age of Eruption

Approximate Age of Eruption ^[53]		
Tooth Type	Primary (months)	Permanent (years)
Central incisor	5–8	6–8
		Delayed tooth eruption can result from genetic disorders or systemic diseases.
Lateral incisor	5–11	7–9
Cuspids	24–30	11–12
First bicuspid	—	10–12
Second bicuspid	—	10–12
First molars	16–20	6–7
Second molars	24–30	11–13
Third molars	—	17–22

Source: Lunt RC, Law DB. A review of the chronology of eruption of deciduous teeth. *J Am Dent Assoc.* 1974;89(4):872–879.



Abnormalities of the Teeth, Pharynx, and Neck



Dental caries (early childhood caries (ECC))



Severe ECC

Dental Caries
Dental caries is a major global health and pediatric problem. White spots on the teeth often reflect early caries. The photographs show different characteristics of caries.



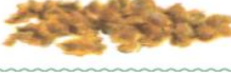
Staining of the Teeth
Various causes can lead to staining of the teeth of children, including intrinsic stains such as tetracycline (left) or extrinsic stains such as poor oral hygiene (cariou lesions shown in previous figures). Extrinsic stains can be removed.

In this age range, likely pathologies

Constipation

- Very common in school aged children
- Withholding behaviors cause back up – rectal distension
- Associated with UTI as well
- Bristol score
- Examine abdomen: can feel stool in LLQ at times
- Ask about elimination
- KUB abdominal film (kidney ureters bladder xray) at times helpful
- Tx: increase fiber rich diet, miralax
- Not to be confused with appendicitis symptoms! - usually in older patients but cannot rule out – EXAM SKILLS MATTER!



THE BRISTOL STOOL FORM SCALE (for children) choose your POO!		
type 1		looks like: rabbit droppings Separate hard lumps, like nuts (hard to pass)
type 2		looks like: bunch of grapes Sausage-shaped but lumpy
type 3		looks like: corn on cob Like a sausage but with cracks on its surface
type 4		looks like: sausage Like a sausage or snake, smooth and soft
type 5		looks like: chicken nuggets Soft blobs with clear-cut edges (passed easily)
type 6		looks like: porridge Fluffy pieces with ragged edges, a mushy stool
type 7		looks like: gravy Watery, no solid pieces ENTIRELY LIQUID

In this age range, likely pathologies

Scoliosis Check

Adams Forward Bend test

Onset variable

Most found in adolescents greater than or equal to 11 yr)

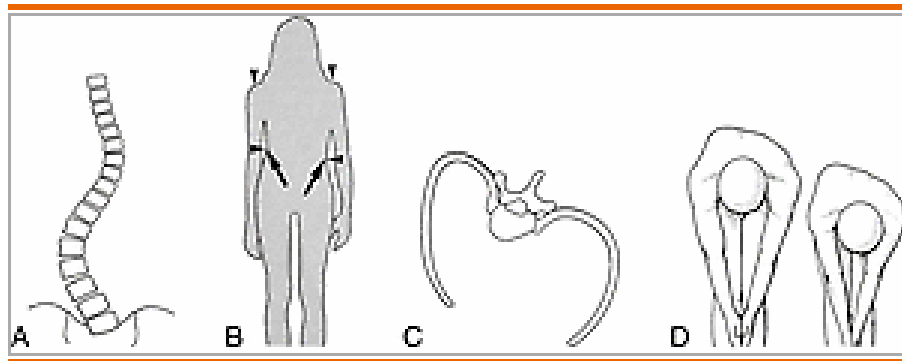
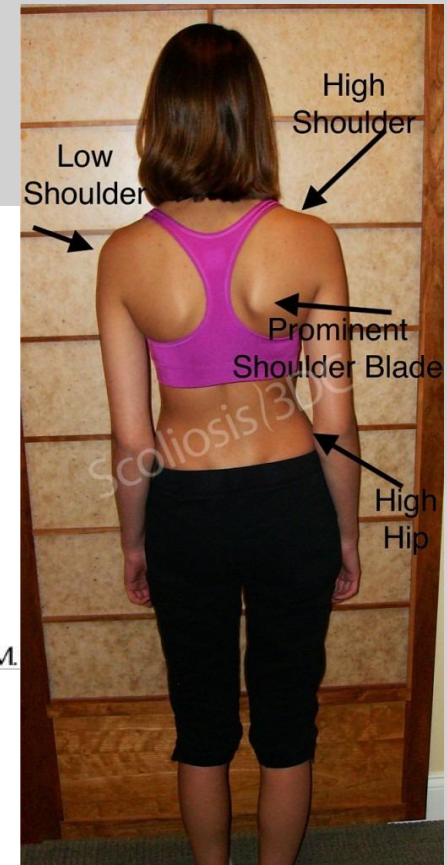


FIG. E1

Structural changes in idiopathic scoliosis.

Tucker Callanan, MD; Eren O. Kuris, MD

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In this age range, likely pathologies

Tanner Staging

Precocious puberty (central vs peripheral)

- All girls younger than 6 years of age with breast or pubic hair development
- Girls younger than 8 years of age with both breast and pubic hair development
- Girls younger than 8 years of age with only breast or pubic hair development: careful history and examination with a bone age determination and close follow-up
- Isolated premature thelarche or premature adrenarche – can be benign
- Boys: onset of secondary sexual characteristics in a boy before age 9

Delayed puberty will be discussed during adolescent lecture



Prepubertal	1	Prepubertal
Breast bud	2	Presexual hair
Breast elevation	3	Sexual hair
Areolar mound	4	Mid-escutcheon
Adult contour	5	Female escutcheon

I	II	III	IV	V

Thank you for watching!!

If any questions, please email me

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Lecture Feedback:

<https://comresearchdata.nyit.edu/redcap/surveys/?s=HXREL4R>



Resources:

1. AAP website
2. CDC
3. Nelson Essentials of Pediatrics
4. The Harriett Lane Handbook Pediatrics
5. Netter's Pediatrics
6. Zitelli and Davis Atlas of Pediatric Physical Diagnosis
7. Pictures from variety of sources including UptoDate